



Sizing Up GLP-1 Impacts: Survey Shows Durable Shift in Pharma, Diets, and Dining

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We recently conducted our 2026 survey of current and former GLP-1 weight-loss medication users and the results are clear: GLP-1 adoption is shifting long-term behaviors affecting pharmaceuticals, food and beverage companies, and restaurants.

These themes were already indicated in our initial [2024 survey](#) of GLP-1s, but today's results are even more striking. Now several years into GLP-1 usage, the survey shows sustained changes in dietary and restaurant behavior that stick even after patients have stopped taking GLP-1s. Following the recent introduction of oral pill versions of GLP-1 medications, the survey showed there is strong interest in orals, largely due to the convenience they offer. However, it also shows there are limits related to affordability and long-term commitment.

The takeaway is that, for pharma, there is a plausible pathway for new oral GLP1s to accelerate market penetration over the coming years. Long-run market expansion is likely, although questions remain about how much consumers will be prepared to pay for orals and how long they will stick with them. Meanwhile, regarding food and beverages, consumers are shifting away from large portions and snacking towards protein-forward, functional, and nutrient-dense choices.

For restaurants, the survey points to declines in the number of times people dine outside the home, partially offset by higher spending each time they eat out. Unsurprisingly, brands with differentiated, healthier offerings or more occasion-based demand are likely to be more competitive.

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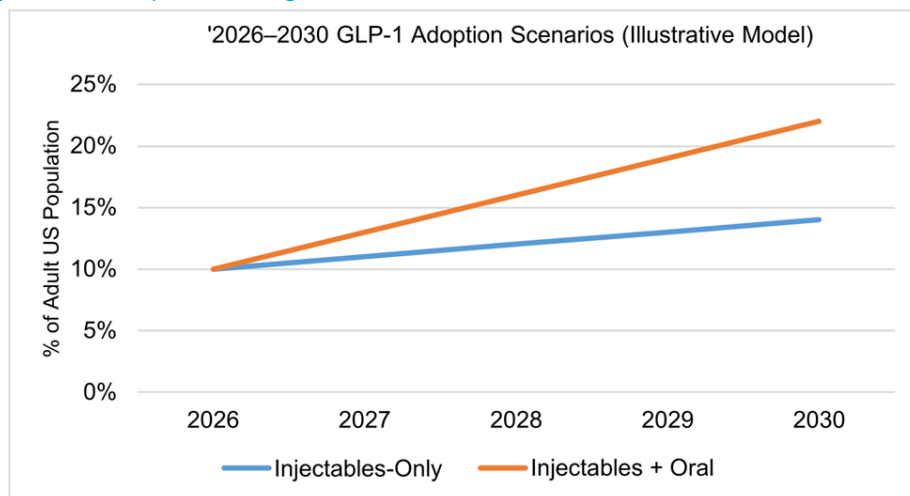
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Figure 1: GLP-1 adoption represents a durable reallocation of consumer demand, in our view; behaviors likely to broaden under accelerated oral GLP-1 adoption through 2030



Source: Deutsche Bank Research

Implications for US and European Pharma

The survey points toward a sustainable promise of oral GLP-1s, but also a cost ceiling, a durability problem, and potential for better risk-benefit to be disruptive. The picture it portrays is of a highly cost-sensitive user base that (still) clearly prefers orals and struggles with adherence to therapy, while mostly intending to return to it at some point and clearly not viewing diet and exercise as a viable alternative.

What that implies for the market outlook is somewhat uncertain: with the up-front caveat that this survey addresses only those trying GLP-1 within the past year and hence perhaps not the key demographic from a market-growth-outlook perspective (i.e., those that have never tried it), for the existing installed user base it implies some risk of injectable cannibalization (though we note this is not particularly apparent yet in prescription data) and also some risk that volume demand may plateau without further price degradation (a minor negative, in our view). However, it also implies that a return to therapy is likely to be high, and improved safety profiles plus oral convenience could help to build out the market opportunity over time. It also suggests the amylin class could help optimize risk-benefit and thus help address the persistent durability problem.

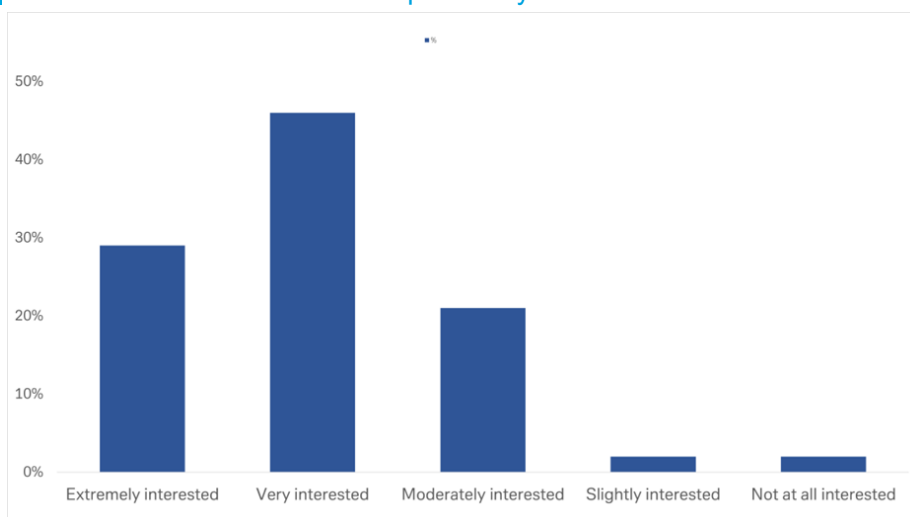
As with our last survey, there was a bias to Ozempic as a preferred brand with a fairly even distribution between Wegovy, Zepbound, and Mounjaro beyond that. Interestingly, the main consequence of increasing use of GLP-1 therapeutics to manage weight loss appears to be a reduction in diet/exercise approaches to helping manage weight loss (rather than the reverse). Beyond that, consistency of adherence appeared static with our last survey (about 70% within a 12-month window), whilst duration of therapy also appeared relatively stable at a mean of 6-9 months. This is despite most patients not being at their target weight loss/implied BMI (current average about 30 versus average target closer to about



25) and many continuing to experience a frequent (20%) rate of GI toxicity of some kind.

Payor coverage did appear to have slightly improved (to about 40%), with most patients still paying up to \$200 per month out of pocket, but interestingly willingness to self-pay has declined to less than half of patients (with most unwilling to pay more than \$200 per month in that scenario). Almost half continue to access therapy via retail pharmacies, with only a modest increase in online channel use. On therapeutic options, interest in oral formulations was very high at about 2/3 (and 80% in younger age groups), with 50% expressing clear preference for orals over injectables all else equal. Interestingly, the greatest sensitivity on the question of switch intent related to cost, with early experience pointing toward a double-digit percentage lower cost for orals versus injectables and most citing this as sufficient to drive switching behavior.

Figure 2: Oral GLP-1 medications are becoming available. With this in mind, please indicate your level of interest in starting to use an oral form should it become an available option for you.



Source: dbDIG Primary Research

The most common reason provided for discontinuation among the approximately 30% who stopped therapy within the past year was cost, though side effects were a close second, and a variety of other reasons followed closely behind. Only about 20% of patients who had discontinued treatment cited "reaching weight loss goal" as the reason. In this subgroup, the advent of oral options also seemed to strike a particular chord (75% extremely or very interested versus 64% in the earlier group of patients still on therapy).

Implications for US and European Consumer Staples

What stood out in our 2026 data compared to our 2024 survey is that GLP-1 adoption is here to stay, with an enduring influence on consumer behavior. The introduction of the oral version of GLP-1s will only reinforce these trends. For packaged food and select beverage categories, the growing prevalence of GLP-1 usage represents a sustained structural pressure that operates through lower



portion sizes, reduced frequency, and selective category substitution rather than outright elimination of consumption.

While many companies have begun to pivot their portfolios in response—introducing protein-fortified snacks, functional beverages, and smaller portion formats—we question whether these innovations can fully offset the net impact of structurally lower intake and frequency. The survey data suggest that many of these shifts improve portfolio relevance and defend share, but do not fully restore lost volume, particularly in indulgent, frequency-dependent categories.

The survey data suggest that protein formats primarily benefit from share reallocation within smaller meals rather than incremental meal creation. Moreover, with traditional categories also chasing protein, we would argue that protein itself is becoming increasingly mainstreamed across everyday food products, diluting the relative advantage of specialized nutrition SKUs and capping category-level growth. Elevated competitive intensity and rising A&P needs increase the risk that theoretical demand alignment does not fully translate into a bigger bottom line.

Outside of performance nutrition, fresh and clean-label portfolios benefit from broader, yet seemingly durable “eat better” behavior rather than calorie avoidance. Survey responses show consistently higher relative consumption of fresh and minimally processed foods during GLP-1 usage, though absolute consumption gains appear incremental rather than transformative.

Flavor-centric and ingredient-led categories appear structurally more resilient than volume-dependent staples. Conversely, sugar-dense, portion-dependent categories face persistent headwinds. Energy drinks, though, stand out as one of the few categories showing net consumption stability or increases among active GLP-1 users. Water and hydration also tend to see benefits, with coffee and tea more neutral.

Overall, GLP-1 adoption increasingly favors goods aligned with satiety, functionality, and quality over calories, but the survey underscores that alignment alone is insufficient. Company-specific execution, competitive position, reinvestment discipline, and cost structure remain decisive in determining whether theoretical portfolio alignment translates into realized financial outcomes.

From a European Food and HPC perspective, our 2026 survey reinforces the view that GLP-1s represent a persistent but comparatively modest influence on consumer demand, rather than a step-change in consumption behavior. In our view, GLP-1 also cannibalizes other forms of weight loss to some degree (though is likely to be more effective and lasting) and as such the net calorie drag per annum that many have worried about seems to be less than feared. It is fair to note though that rolling 12m US Nielsen aggregate “Food” volumes are running down about 1% year-over-year currently and the pace is accelerating. It’s difficult to disaggregate GLP-1 from macro, demographic, SNAP and immigration factors, however, its unlikely to be a non-zero impact.

Importantly, the 2026 cohort appears to have entered GLP-1 usage with healthier baseline habits than in 2023, resulting in a smaller incremental behavioral shift once on medication. In several categories, the change from “before” to “during” GLP-1 usage is less pronounced than previously observed, suggesting an



element of normalization rather than acceleration. That doesn't make the GLP-1 effect itself less valid in its net effect, just that it may not all be related to actual GLP-1 usage. We have also previously noted that the US has also gone through waves of weight loss previously, it's not linear and as such GLP-1 usage increased at a likely time of one of these waves persisting post COVID anyway and we have previously seen a pivot to categories like dairy during non-GLP-1 weight loss.

While some behavioral persistence is evident post-discontinuation, the data do not point to a broad-based collapse in category participation. In fact, select traditionally "indulgent" categories show stabilization—or modest re-engagement—among former users, complicating a simple winners/laggards framework. At an aggregate level, the data are also consistent with lower intensity rather than outright avoidance, implying moderation rather than displacement and an opportunity for companies to tailor their suite of SKUs to mitigate or indeed benefit. As of yet we have seen limited impact on demand for household and personal care categories; thus far, consumer health companies have seen a surprisingly limited number of innovations that support GLP-1 usage.

For our European Beverages coverage we view GLP-1 as a headwind to consumption overall but see some of the survey responses as providing conflicting data points to this potential risk. We view GLP-1 as one of a number of threats to beverage alcohol consumption including 'health', younger consumers, cannabis and, for spirits, smaller players and ready-to-drink. Companies with a higher exposure to the US and those geographies where GLP-1 is coming off patent in the near future (Canada, India, China, Brazil) likely present the greatest risk with beverage alcohol more 'at risk' than soft drinks.

The impact on nicotine consumption is not specifically covered in the survey results. We do recognize that there is growing body of scientific evidence to support the role GLP-1s may play in tackling smoking addiction, but efficacy varies by the specific drug used. The observational data also point to increased effectiveness in reducing nicotine consumption (cigarettes and vapes) although these studies remain relatively small in scale. The industry has long dealt with declining volumes, and so we see GLP-1's as another contributor to the volume contraction but one which, on current evidence, does not represent an existential threat to the industry.

We would also posit the view that the motivation for smoking cessation is typically health related. Increasingly, the industry is able to offer consumers a broad range of smoking alternatives via Next generation products, which significantly improve the health outcomes, compared with smoking.



Figure 3: Categories emerging as relative beneficiaries with the increasing use of weight loss medication

	Category	Description/Details
Relatively Advantaged Categories	Protein shakes and bars	Protein shakes and bars are clear winners. In 2026, 72% of current GLP-1 users consume more than before, up from 64% in 2023, reinforcing protein formats as efficient meal substitutes rather than incremental calories
	Fresh meat and fish	Fresh meat and fish benefit from satiety-led substitution. In 2026, 59% of current users report eating more than pre-medication, up from 45% in 2023, supporting increased prioritization of whole, minimally processed protein sources.
	Fresh produce	Fresh fruits and vegetables remain durable beneficiaries. In 2026, 73% of current GLP-1 users report eating more produce, similar to 2023, indicating sustained "eat better" behavior despite overall moderation in total food intake
	Nuts	Nuts show low downside and solid upside. In 2026, 56% of current users report eating more nuts than before, positioning the category as a functional, nutrient-dense snack aligned with satiety rather than indulgence.
	Condiments, sauces, and spices	Condiments are resilient but not growth-oriented. In 2026, "a lot less" consumption is only 12% while "neither more nor less" rises to 57%, suggesting stability as flavor intensity per bite matters in smaller meals. size.
	Energy drinks	Energy drinks stand out within beverages. In 2026, 47% of current users report drinking more than before, while only 33% report drinking less, supporting functional, zero-sugar positioning over indulgent beverage formats.

Source: Deutsche Bank Research

Figure 4: Categories impaired by GLP-1 usage but more resilient, especially with adaptation to remain competitive

	Category	Description/Details
Mixed / Execution-Dependent Categories	Packaged and frozen meals	Frozen and refrigerated meals sit at the execution frontier of GLP-1 impact: neutrality remains the modal outcome, with incremental—not structural—pressure, hinging on portion control, protein density, and clearer health cues
	Canned foods and traditional pantry staples	Pantry staples are neutral-to-slightly pressured, not broken: consumption skews toward "same or less," with limited upside absent reformulation, protein enrichment, freshness cues, and improved portion transparency
	Coffee	Coffee is compositionally resilient rather than volume-challenged: core consumption holds steady, while sugar- and syrup-heavy formats decline—shifting pressure to sweetened customization, not coffee occasions themselves
	Tea	Tea is one of the most resilient categories in the dataset: high neutrality, minimal cutbacks, and muted intent-to-reduce reinforce its role as a low-calorie, habitual utility beverage insulated from GLP-1 moderation dynamics

Source: Deutsche Bank Research



Figure 5: Categories that are more adversely impacted by the use of weight loss medication

	Category	Description/Details
More At-Risk Categories	Chocolate and candy	Chocolate and candy face the clearest structural pressure. Roughly half of current GLP-1 users consume less in 2026, and most former users fail to rebound, confirming sustained demand erosion in calorie-dense indulgence
	Cookies and biscuits	Cookies and biscuits remain highly exposed. One-third of current users report eating "a lot less" in 2026, while ~75% of former users continue to eat less or stop entirely, reflecting vulnerability of habitual, low-satiety snacks
	Ice cream	Ice cream participation is persistently impaired. About 70% of current users report same or lower consumption, and a similar share of former users continue to eat less or none, signaling weak post-GLP-1 recovery
	Salty snacks	Traditional salty snacks face uneven but meaningful pressure. Chips consumption declines, with "a lot less" rising to 17% in 2026; pretzels show limited rebound, reinforcing structural headwinds for refined-grain snack formats
	Cereals and refined pantry staples	Cereals show elevated "eat less" signals during GLP-1 usage and weak normalization afterward. Their association with refined carbohydrates and low satiety leaves cereals structurally disadvantaged versus protein-dense alternatives
	Alcohol (Beer, Wine, Spirits and RTD cocktails)	Alcohol faces moderate but notable headwinds. Participation declines materially (spirits, beer, wine), yet "drink more" exceeds "drink less" during treatment, suggesting reduced frequency rather than category exit and a nuanced, uneven risk profile

Source: Deutsche Bank Research

Implications for Restaurants

The increasing penetration of GLP-1 weight loss drugs is intuitively a headwind for total restaurant spend, and our survey results are in line with this thinking. Despite the fact that many users eventually stop taking GLP-1s, they continue to show sustained changes in behaviors. Our survey results indicate that former GLP-1 users do not meaningfully return to full-service restaurants after taking the medication, though some return to fast food and coffee shops at a rate above current GLP-1 users, and in reality, we believe restaurant trends rebound more than our survey suggests. This is based on anecdotal commentary and behaviors seen in other categories. The read-through on individual frequency was more positive, with current GLP-1 users reporting an increase in visits to full-service restaurants, and a modest decline in visits to fast food, coffee shops, and takeout relative to pre-GLP-1 levels. Moreover, average spending on meals per visit was higher than baseline, suggesting a more tempered impact on sales compared to traffic.

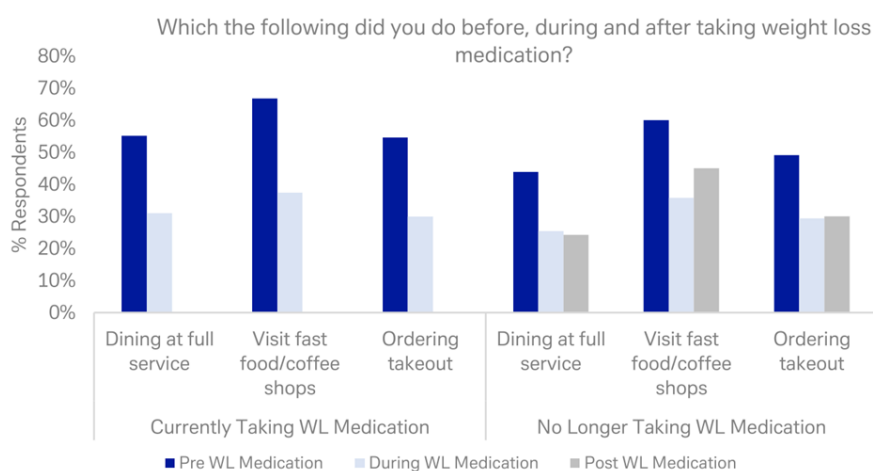
Restaurants have started to address behavioral shifts by offering smaller portion sizes, protein-forward options and functional beverages. We believe these innovations can help more effectively sustain participation in the restaurant industry; brands that are differentiated, screen healthy, and are more occasion based are relatively best positioned to be competitive.

It is clear that overall food consumption decreases when consumers are on GLP-1 medications, though the implications once consumers transition off are less clear. Based on our survey, 35% consumers report eating more on GLP-1 medications, 24% eat about the same, and 41% report eating less, for a net 6% eating less. When consumers were asked about current eating habits relative to pre-GLP-1 medication, among those currently taking GLP-1s, a net 10% reported eating less compared to pre-medication (38% more, 48% less). Notably, among consumers that are no longer taking GLP-1s, when compared to prior behaviors, a net 20% indicated they are actually eating more (45% more, 24% less), suggesting former users revert toward prior aggregate consumption levels.



That said, there is still a change in how consumers eat, most notably an increase in cooking. Based on our survey, among consumers currently taking GLP-1s, a net 54% of consumers are cooking more at home. Cooking usage also remains elevated among consumers no longer taking GLP-1s, with a net 47% of consumers cooking more at home than before medication.

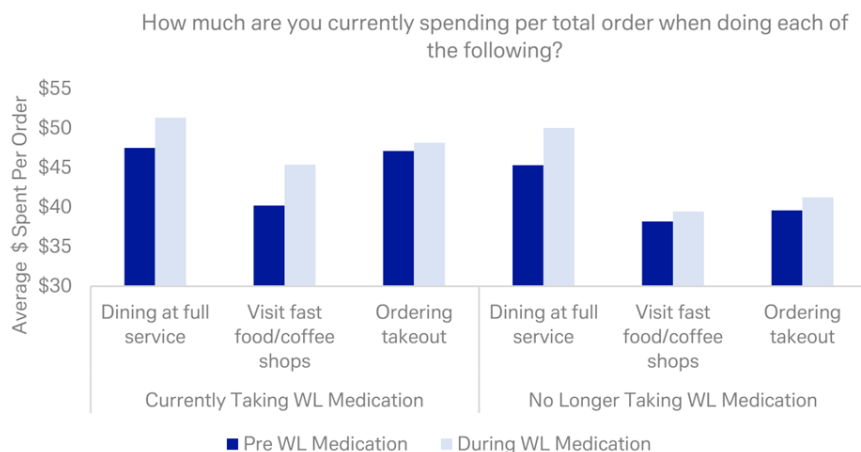
Figure 6: Our survey shows a meaningful and broad-based decline in restaurant usage when consumers take GLP-1s. Among current GLP-1 users, the percentage of respondents that visit restaurants drops by 25-30pp versus pre-weight loss behavior. Among former GLP-1 users, the percentage of respondents that visited restaurants while taking these medications versus prior behavior decreases at a relatively similar 20-25pp.



Source: dbDIG, Deutsche Bank Research

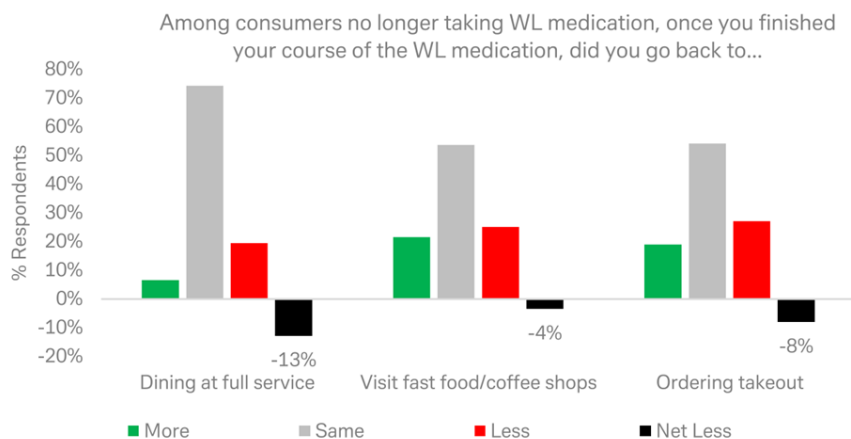


Figure 7: Among GLP-1 users that regularly visit restaurants, average spend per month did not take much of a hit, noting both current and former GLP-1 users report higher average spend per visit for full service, fast food/coffee shops and takeout. It is unclear whether the higher average spend per visit is a reflection of perceptions around higher prices or consumers ordering more/trading up.



Source: dbDIG, Deutsche Bank Research

Figure 8: Individual consumption for former GLP-1 users that are still eating at restaurants seems reasonably resilient (or at least manageable). On a net basis, 13% of consumers said they were dining less at full service compared to pre-GLP-1 behaviors, 4% said they were visiting fast food/coffee shops less, and 8% said they were ordering takeout less.



Source: dbDIG, Deutsche Bank Research



The Bottom Line: A Lot to Digest

Our survey implies that GLP-1s are starting to look less like a therapeutics cycle and more like an enabling platform. It is reshaping how consumers manage weight, what they eat and drink, and whether and how they choose to dine out. For pharma, orals offer an expanded market, offset to an extent by affordability and other constraints. For consumer staples and restaurants, brands that can focus on health-forward innovations will remain more competitive in this new environment.

Research Methodology

This study focuses on both current and past GLP-1 users, with demographic data collected via a dedicated set of questions in our US monthly survey. Compared to the 2023 wave, several demographic shifts were observed:

Gender: The user base is becoming more gender-balanced, with male GLP-1 users increasing from 40% to 45%.

Income: While the overall income distribution remained stable, there was a marginal increase in GLP-1 users within Income Tier 1 (households with an income up to \$25,000).

Overall, the survey was conducted through dbDIG from March 20-30, 2026, with 550 respondents. We would like to thank Anthony Chaimowitz for his significant contribution to the survey and this report.



Appendix 1

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